INFORMATION TO KEEP WITH YOU

# My complete treatment handbook

## Regorafenib (Stivarga) monotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

**Displayed calendar**
**28 days**
See the active treatment phase

**Treatment stages**
**1**
Explained in the treatment reference

**Regimen source**
**NCCP 00244**
Version 5

> Three tablet weeks, then one tablet-free week

## Treatment stages

01 **21 tablet days, then seven rest days**
28-day cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

## Inside your handbook


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**My name** _____

**Hospital / oncology team** _____

> Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

# Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

## When to get help

**Emergency: call 112 / 999**

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

**Contact your oncology service immediately**

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever, shivering, sudden illness, yellow eyes or skin, dark urine, severe or persistent abdominal pain, black or bloody stools, new bleeding, severe diarrhoea or inability to drink. Severe chest pain, major breathing difficulty, heavy bleeding, collapse, seizures, or a severe headache with confusion or visual change need emergency help: 112 / 999.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

## Your targeted medicine and what it does

Regorafenib is a targeted tablet treatment for colorectal cancer that has spread, after previous treatments or when those treatments are unsuitable. The aim is to control the cancer. Your team discusses the likely benefit, side effects, alternatives and your priorities.

ORAL MULTIKINASE INHIBITOR

### Regorafenib

Stivarga



**Blocks several signals involved in tumour growth and blood-vessel formation**

Regorafenib blocks several protein switches called kinases. These help cancer cells grow and help blood vessels support a tumour. The medicine also affects normal tissues, so skin, liver, blood pressure and other symptoms need close review.

Growth and blood-vessel signals → Regorafenib blocks several kinases → Tumour-supporting signals are reduced

**What to watch for**

Report sore hands or feet early. Attend liver and blood-pressure checks even if you feel well, and get urgent help for serious new symptoms.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

## What this treatment is for

Confirm the diagnosis, treatment setting and individual goal.

### Evidence and uncertainty

Median overall survival was 6.4 months with regorafenib and 5.0 months with placebo in the trial. These are group medians, not a prediction of your lifespan or a guaranteed extra 1.4 months. Side effects were more frequent with regorafenib. The team should relate the evidence to your health, previous treatment and priorities.

### How long it continues

There is no fixed total number of cycles. Treatment continues while benefit and tolerability justify it. Your team reviews symptoms, blood tests, scans and your preferences; it may pause, reduce or stop the medicine.

## Evidence for previously treated colorectal cancer

CORRECT compared regorafenib plus best supportive care with placebo plus best supportive care in 760 people with metastatic colorectal cancer after standard treatments.

CHAPTER 02

# Your schedule, tests and appointments

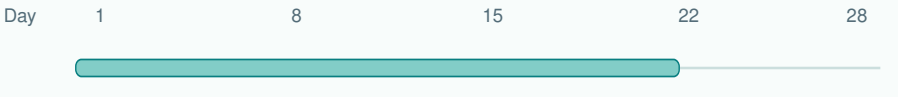
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

## A 28-day tablet cycle

**Regorafenib (Stivarga) monotherapy**

28-day view




This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

**28-DAY CYCLE**

### 21 tablet days, then seven rest days

D1–21 D22–28

Take the prescribed dose once daily on Days 1–21, then take no regorafenib on Days 22–28. Each cycle lasts 28 days. Take tablets at the same time after a light, low-fat meal. Start another cycle only when the team authorises it.

**BASELINE → TREATMENT → FOLLOW-UP**

## Checks before, during and after treatment

Some tests are required for everyone; electrocardiogram (ECG)/QT, pancreatic enzymes, international normalised ratio (blood-clotting test) (INR) and extra checks depend on risk, symptoms or other medicines. There is no single mandatory imaging or tumour-marker interval for all patients.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00244 v5 · Regorafenib Monotherapy · revised 25 March 2025 · EMA Stivarga · current product information and patient leaflet · checked 5 November 2026

## Tests in your plan

### Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

**For a colorectal cancer plan, if your team uses CEA; your team sets the interval.**

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

# Regorafenib (Stivarga) monotherapy: treatment and checks

## Previously treated metastatic colorectal cancer

Take the prescribed dose once daily on Days 1–21, then take no regorafenib on Days 22–28. Each cycle lasts 28 days. Take tablets at the same time after a light, low-fat meal. Start another cycle only when the team authorises it.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

### Treatment, tests and reviews across the course

| Care / check | Before starting | First six weeks | Up to two months | Later cycles | After / stopping |
|---|---|---|---|---|---|
| **Liver blood tests** NCCP 00244 | Baseline liver profile | Every two weeks | Every two weeks | Monthly or as clinically indicated | Follow any abnormal results until reviewed / resolved |
| **Blood pressure** NCCP 00244 | Baseline; control existing hypertension | Weekly | Before each cycle after week six; extra if needed | Before each cycle or as indicated | Follow the team's plan; do not stop blood pressure (BP) tablets yourself |
| **Blood count and kidneys** NCCP 00244 | Baseline full blood count (FBC) and renal profile | Regular tests as arranged | Continue scheduled monitoring | Regular tests as arranged | Review residual blood-count, fluid or kidney problems |
| **Thyroid and additional tests** NCCP 00244 | Thyroid function; ECG/QT if at risk; pancreatic enzymes if indicated | Thyroid / pancreas tests if indicated; ECG / heart-rate review if needed | If on warfarin: close INR checks | Further tests according to symptoms and medicines | Follow abnormalities and symptoms individually |
| **Symptoms and cancer response** NCCP 00244 | Agree aim, baseline symptoms and review plan | Review hands/feet, mouth, bowel, appetite, energy and wounds | Review tolerability and any interruptions | Scans / markers according to the individual cancer plan | Agree follow-up, next treatment or supportive care |

### What each check is for

1



### 21 tablet days, then seven rest days

28-day cycle

Regorafenib is a targeted tablet treatment for colorectal cancer that has spread, after previous treatments or when those treatments are unsuitable. The aim is to control the cancer. Your team discusses the likely benefit, side effects, alternatives and your priorities.

**Ask** / **act:** Confirm the current phase, prescription, tests and support medicines.

### At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

## Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

### Prescription and tablet calendar

Confirm the exact daily tablet number, Days 1–21 and Days 22–28, blood-test appointments and who authorises the next cycle. A clinician may prescribe a lower starting dose or change it after side effects; follow the issued prescription.



STEP 2

### Taking tablets with food

Swallow the prescribed tablets whole with water at the same time each day after a light meal containing less than 30% fat. Ask the pharmacist for a practical meal example. Follow your own tablet label and calendar; do not change the dose yourself. Avoid grapefruit juice.



STEP 3

### Missed or vomited doses

If you miss a dose, take it as soon as you remember on the same day. Do not take two doses in one day or make up the dose the next day. If you vomit after taking regorafenib, do not take extra tablets; tell the team. If unsure, contact the pharmacist or oncology team.

STEP 4

### Storage and handling

Keep tablets in their original bottle, tightly closed, away from children and protected from moisture. Record when the bottle is first opened; the leaflet says to discard remaining tablets seven weeks after opening. Ask the pharmacist about safe disposal and help if you cannot swallow whole tablets.



STEP 5

### Accidental extra tablets

Seek medical advice straight away if you take more than prescribed. Take the pack and current calendar with you.

STEP 6

### Visits and practical support

This is tablet treatment without an infusion or take-home pump, but it still needs regular visits and tests. Ask who arranges appointments and results, and tell the team about travel, work or caring needs before changing the schedule.

CHAPTER 03

# Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

## Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

| Treatment or check | Timing in this view |
|---|---|
| **Regorafenib tablets** |  **Days 1–21** Once daily after a light, low-fat meal |
| **Tablet-free week** |  **Days 22–28** No regorafenib; review recovery and the next prescription |
| **Symptoms and scheduled tests** |  **Days 1–28** Report illness on treatment days and rest days |

**Before the first tablet:** Confirm consent, baseline tests, pharmacy teaching, the light-meal routine, skin care and working urgent contact numbers.

**Longer-term review:** There is no fixed final cycle. Review benefit and side effects with the team and agree what follows if treatment stops.

**Early weeks have extra checks:** The early liver and blood-pressure schedule continues across tablet-free days. A calendar stamp or a tablet supply does not establish that treatment is safe to restart.

> **Why these bands stay broad:** The bars show the prescribed tablet and rest days. They do not predict when side effects will happen or establish safe days. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00244 v5 · Regorafenib Monotherapy · revised 25 March 2025 ↗

> **There are no guaranteed "safe days".** Use your oncology unit's urgent contact plan throughout the cycle, including tablet-free days. Symptoms can need attention before the next blood test or appointment.

## Between-treatment notes

### Day 1–7: Start with a clear calendar

Take the prescribed once-daily tablets after a light, low-fat meal. Early blood-pressure checks and a hand/foot care plan are important. Confirm the first blood-pressure and liver-test appointments before leaving.

### Day 8–21: Continue tablets and early monitoring

Keep taking the prescribed dose unless the team changes it. Symptoms may build over time; hands, feet, appetite, bowel symptoms and energy need review. Report new problems early; do not wait for the end of the tablet block.

### Day 22–28: Tablet-free week

Take no regorafenib. Side effects and abnormal tests can persist during the break. The next cycle may need a delay or a different dose. Complete arranged tests and confirm the next prescription and start date with the team.

## Support around the medicines

### 1 Hands and feet

Inspect your hands and feet regularly and tell the team about pain, redness, swelling or blisters early. Ask for the unit's skin-care plan; it may include moisturising creams and protection or cushioning for hands and feet. Do not wait until walking or everyday hand use becomes difficult.

### 2 Sickness and bowel support

Regorafenib has minimal-to-low oral sickness risk; routine premedication is not usually required. Agree any anti-sickness and diarrhoea medicines with the team. Repeated vomiting, diarrhoea, poor intake or reduced urine needs prompt advice, even if you already have supportive medicines.

### 3 Mouth care, food and energy

Report mouth soreness, poor appetite, weight loss or fatigue affecting your usual activities. Ask about mouth care, nutritional help and a manageable activity/rest plan. Take the tablets after a light, low-fat meal and drink as advised for your health.

### 4 Blood pressure and liver checks

Attend tests even when you feel well. Liver tests are every two weeks for the first two months, then monthly or as clinically indicated. Blood pressure is checked weekly for six weeks, then before each cycle or as indicated. Confirm who reviews each result.

### 5 Medicines and procedures

The pharmacist needs a complete medicines and supplements list. Avoid grapefruit juice. Report planned surgery, dental procedures and wounds because treatment can affect healing. Tell the team if you take warfarin; closer INR checks are needed.

### 6 Pregnancy, fertility and feeding

Discuss fertility before treatment. Women who could become pregnant and men should use effective contraception during treatment and for at least eight weeks afterwards. Do not breastfeed during treatment. Contact the team promptly if pregnancy is possible.

## Your tablet and rest-day plan

Use the written prescription and calendar.

### Extra tablets or swallowing difficulty

If you accidentally take too much, get medical advice straight away. Ask the pharmacist for help before altering or crushing a tablet.

## Your tests and who follows them

Feeling well does not replace the early monitoring schedule.

### Before treatment

Blood count, kidney/liver tests, thyroid function and blood pressure are checked. ECG/QT is used for patients at risk; pancreatic enzymes are measured when clinically indicated.

### During treatment

Baseline blood count, kidney/liver profile, thyroid function and blood pressure. ECG/QT if at risk and pancreatic enzymes if indicated. During treatment: regular blood count and renal profile; liver profile every two weeks for two months, then monthly or as indicated; BP weekly for six weeks, then before each cycle or as indicated. Thyroid, pancreas and cardiac checks as indicated; close INR monitoring with warfarin. Scans and markers follow the Consultant's individual plan.

### After a pause or after stopping

The team decides when to repeat abnormal tests and how to follow persistent side effects. Ask for the next review date and who remains your contact. Do not restart after a treatment hold without explicit instructions.

### Scans and tumour markers

Your Consultant sets disease-assessment tests and timing for your situation. This regimen does not impose one fixed scan or tumour-marker timetable.

## Skin, eating and daily activities

Early reporting can help prevent symptoms from becoming harder to manage.

### Thyroid, voice and other changes

Tell the team about worsening tiredness, feeling unusually cold, a hoarse voice, numbness or tingling, cramps or other persistent changes. The medicine can affect thyroid function, nerves and blood salts; symptoms may need tests or supportive treatment.

### Driving and work

Do not drive or use machinery if symptoms affect concentration or reaction time. Discuss fatigue, painful hands/feet and your work or caring responsibilities with the team.

## Symptoms needing urgent help

Frequency and urgency are different: a rare effect can still need immediate care.

### Liver warning signs

Yellow skin or eyes, dark urine, or new confusion needs urgent medical advice. Liver injury can also start without obvious symptoms, which is why scheduled blood tests matter.

### Bleeding, bowel problems and dehydration

Get urgent help for black or bloody stools, vomiting or coughing blood, severe or persistent abdominal pain, significant new bleeding, repeated vomiting or diarrhoea, or very little urine. Heavy bleeding or collapse needs 112 / 999.

### Heart and neurological symptoms

Call 112 / 999 for chest pain with breathlessness or fainting, severe breathing difficulty, seizures, or a severe headache with confusion or visual changes. Do not wait for a routine blood-pressure appointment.

### Infection

Fever, shivering or suddenly feeling unwell needs immediate oncology advice. New painful urination, a severe cough, shortness of breath, a sore throat or a red painful area can also signal infection.

### Wounds, vessels and severe skin reactions

Tell the team about an unhealed wound or planned procedure. Seek urgent help for a widespread painful or blistering rash. New severe pain, swelling, neurological symptoms or sudden deterioration may need emergency assessment; the medicine can rarely affect blood-vessel walls or small blood vessels.

## Other medicines and procedures

A complete medicines list helps the pharmacist check interactions.



### Medicines and procedures



### Include non-prescription products

Tell the pharmacist about antibiotics, antifungals, epilepsy medicines, anticoagulants, pain medicines, supplements and herbal remedies, particularly St John’s wort. Ask before starting or stopping anything. Avoid grapefruit juice.

### Allergies and ingredients

Tell the team about medicine allergies, including reactions to similar medicines. Stivarga contains sodium and soya-derived lecithin; ask the pharmacist if these affect your care.

## Pregnancy prevention, storage and handling

Agree personal precautions before the first dose.

## Your choices and practical support

You can ask for clarification and reconsider treatment.

### Consent and alternatives

Discuss the colorectal disease-control aim, likely benefit and uncertainty, alternatives including supportive care without regorafenib, early hand-foot skin reaction, blood-pressure and liver monitoring, fatigue, diarrhoea and appetite loss, serious bleeding/infection/heart/bowel risks, wound healing, interactions, pregnancy prevention and the option to reconsider. Agree who authorises changes and restarts.

### Keep an urgent contact plan

Carry the treatment card and your current medicine list. Record the actual daytime and out-of-hours oncology numbers. Show the card when seeing another clinician and explain when you last took regorafenib.

### A printable record

Use the passport to record prescribed changes, symptoms, tests and questions. It supports the conversation; it does not authorise tablets or replace the prescription.

## Questions worth asking

**Does having more side effects mean treatment is working?**

No. Side effects do not reliably show whether the cancer is responding. Report them so they can be treated. Your team assesses benefit using the tests and review plan appropriate to your cancer.

**Why might a dose be delayed or reduced?**

Your team may allow recovery, investigate a symptom or reduce the chance of lasting harm. Ask which drug changes, why, and when review is planned. A delay does not itself mean treatment has failed.

**What should I say when calling?**

Give the regimen name, last treatment date, symptoms and when they started. Explain whether you can eat, drink, pass urine and manage usual activities. Give your temperature if available, but do not delay calling to collect details.

### What belongs in the consent conversation?

The individual aim and likely benefit, uncertainty, reasonable alternatives including not proceeding, common and serious risks, lasting effects, fertility and practical concerns. You can ask for an interpreter, more time or another explanation and can reconsider your decision.

### How many cycles should I use this passport for?

The passport provides reusable recording pages, not a prescribed course length. Your oncology team sets the duration and can provide extra pages.

### Can I use this page to decide if blood results are safe?

No. Treatment decisions require the clinical protocol, symptoms, previous toxicity, organ function and the prescriber's review. The patient page explains the checks and does not authorise treatment.

### Does "targeted" mean there are no serious side effects?

No. The medicine blocks cancer-related signals but can also affect healthy tissues. Liver tests, blood-pressure checks and prompt symptom reporting are essential.

### Can I use a diary stamp to decide to restart?

No. A stamp records a visit or completed cycle. Your team must confirm the dose and start date, especially after an interruption or abnormal test.

### Why are checks so frequent at the beginning?

Blood pressure and liver tests can change before you notice symptoms. The early monitoring schedule helps the team identify problems and decide whether treatment needs to change.

### Do I need radiotherapy or an infusion with these tablets?

This pathway is regorafenib monotherapy. An additional cancer treatment would require its own agreed plan and safety discussion.

### Are the seven rest days safe days?

They allow a break from tablets, but problems can continue or develop then. Keep using the same urgent contact plan.

> **Try explaining the plan back:** Can you show the 21 tablet days and seven rest days, explain the meal and missed-dose instructions, identify the early liver and blood-pressure checks, and say whom you will call if unwell?

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**Treatment scope · NCCP 00244a** This guide covers regorafenib for previously treated metastatic colorectal cancer. Regorafenib is also used in other cancers, but their eligibility and evidence are different. Confirm the treatment setting on your prescription.

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Regorafenib oral cycle: Days 1–21, then Days 22–28 off

**Active medicines:** Regorafenib

**Emetogenic risk:** Minimal-to-low oral

| Medicine | Dose / route | When |
|---|---|---|
| Rescue antiemetic | Individual prescription under the oral-systemic anticancer therapy (SACT) pathway | As required, not routine daily prophylaxis<br>NCCP V6 Table 5 offers metoclopramide, prochlorperazine or one 5-HT3 antagonist. Choose and prescribe one appropriate option after contraindication, interaction, renal-function and QT review; do not combine alternatives automatically. |

### Additional phase-specific support

- No routine premedication is usually required. Provide local hand–foot skin reaction prevention and treatment; examine pressure areas and report pain, blisters or impaired walking early.
- Take the prescribed dose once daily after a light meal containing less than 30% fat, as clarified by current Stivarga SmPC; do not replace a vomited dose or double the next day.
- Check liver tests every two weeks for the first two months, then monthly or as clinically indicated; abnormalities require the separate source-specific closer monitoring plan.
- Check blood pressure weekly for the first six weeks, then before each cycle or as clinically indicated. Baseline thyroid testing; subsequent thyroid, lipase and amylase checks as clinically indicated.
- Review strong CYP3A4/UGT interactions, bleeding, infection, ischaemia, wounds and planned surgery; close international normalised ratio (blood-clotting test) (INR) monitoring if taking warfarin.
- Dose changes require consultant review. HFSR recovery, occurrence count and previous dose determine the pathway; permitted re-escalation is not a patient-led action.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

# What to watch for

You do not need to know which medicine caused a symptom before calling.

### 1 Head / energy

**Regorafenib**

Fatigue can affect daily life. Severe headache with confusion, visual change or a seizure needs emergency help.

### 3 Heart / blood pressure

**Regorafenib**

Blood pressure can rise without symptoms. Attend checks; chest pain, fainting or severe breathlessness needs emergency help.

### 5 Liver

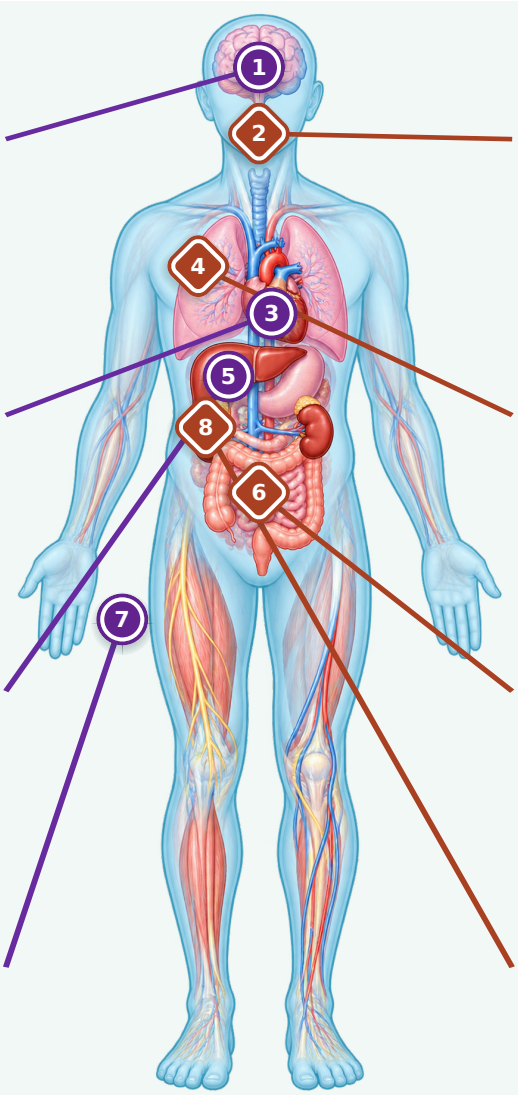
**Regorafenib**

Liver injury may first appear in blood tests. Yellow skin or eyes, dark urine or confusion needs immediate advice.

### 7 Hands / feet / skin

**Regorafenib**

Pain, redness, blisters or thickened skin can limit walking or hand use. Report early and follow your skin-care plan.




### 2 Mouth / appetite

**Regorafenib**

Soreness, ulcers, taste changes or poor appetite can affect eating. Report difficulty drinking or weight loss.

### 4 Blood / infection

**Regorafenib**

Bleeding, bruising or infection can occur. Fever or sudden illness needs urgent advice; heavy bleeding needs emergency help.

### 6 Bowel / abdomen

**Regorafenib**

Diarrhoea can cause dehydration. Severe or persistent pain, black stools or blood in stools needs urgent help.

### 8 Kidneys / fluids

**Regorafenib**

Vomiting or diarrhoea can reduce fluid intake and urine. Very little urine or inability to drink needs urgent advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology immediately for fever, shivering, sudden illness, yellow eyes or skin, dark urine, severe or persistent abdominal pain, black or bloody stools, new bleeding, severe diarrhoea or inability to drink. Severe chest pain, major breathing difficulty, heavy bleeding, collapse, seizures, or a severe headache with confusion or visual change need emergency help: 112 / 999.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

CHAPTER 04

# Sources and further information

## IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.


1. **National Cancer Control Programme (NCCP) 00244 v5 · Regorafenib Monotherapy · revised 25 March 2025**
   https://healthservice.hse.ie/documents/6347/244_v5_Regorafenib_Monotherapy.pdf
2. **EMA Stivarga · current product information and patient leaflet · checked 5 November 2026**
   https://www.ema.europa.eu/en/documents/product-information/stivarga-epar-product-information_en.pdf
3. **Grothey et al. CORRECT · colorectal regorafenib monotherapy · Lancet 2013**
   https://pubmed.ncbi.nlm.nih.gov/23177514/
4. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq
5. **NCCP antiemetics v8, Table 1 pp4–5**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf